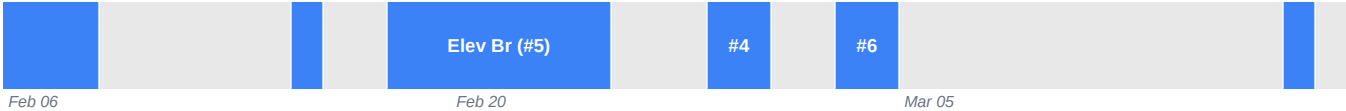


Patient ID: SallenPeriod: February 6 to March 18, 2024Report Date: March 19, 2024Coverage: 676/993h (53.7%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 42 days from Feb 06 to Mar 18

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
24	25h	Very High Breathing	<1	68%

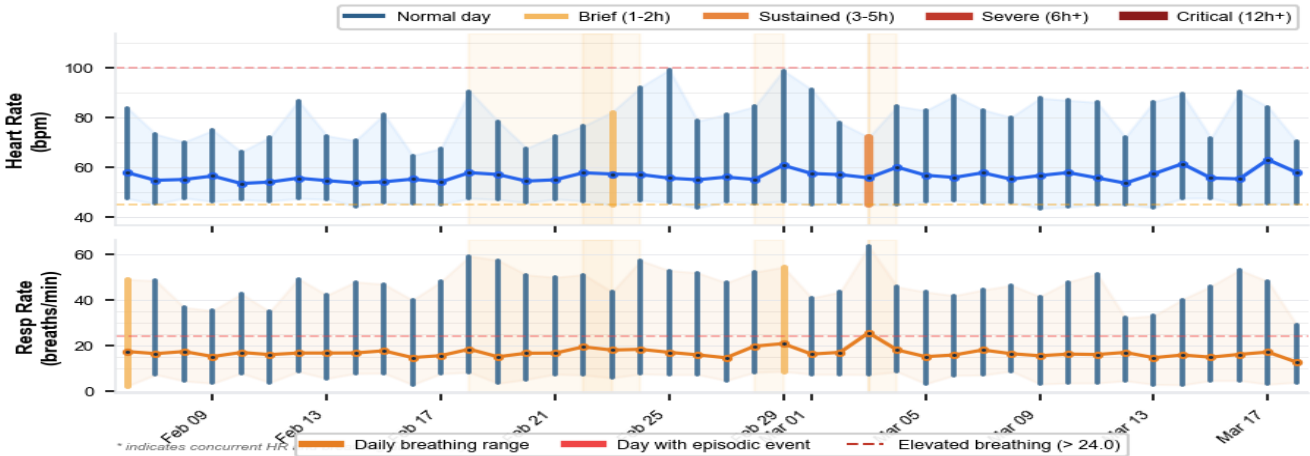
Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Very High Breathing	63 brpm	2h	1h	2	26 brpm	Mar 03
2	High Breathing	43 brpm	2h	1h	1	19 brpm	Feb 22 to Feb 23
3	High Breathing	45 brpm	1h	1h	1	26 brpm	Mar 03
4	Elevated Breathing	54 brpm	6h	2h	2	20 brpm	Feb 28 to Feb 29
5	Elevated Breathing	57 brpm	8h	1h	1	17 brpm	Feb 18 to Feb 24
6	Elevated Breathing	43 brpm	2h	1h	1	22 brpm	Mar 03 to Mar 04

RR values outside physiologic range (6–60 breaths/min).

Clinical Pattern Observations:

- 1. Breathing variability: RR spread 11 brpm (10 to 22).
- 2. Clustered pattern: Episodic events on 4 of 42 days (9%).



Red bars indicate days with detected episodic events.
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.